Psychiatric Evaluation
Patient: [redacted]
Date of birth: [redacted]
Date of Evaluation Updated: 1/5/2019

History of presenting illness: [redacted] old [redacted] Man presents with depression, anxiety and PTSD. This evaluation is based on three sessions between the patient and the writer. July 11th, July 25th, August 15 and August 29th, September 21st 2018, October 31st and December 19th 2018. The patient will follow up with me for medication management and psychotherapy twice monthly. Please consider this as a treatment plan as well.

The patient reports that his panic attacks and depression have been worsening for the past 3 months and specifically have been worsened by the actions of his wife and her family. The patient and his wife are separated but still legally married. The patient reported that are several events between him, his wife and her family that may have contributed to the patient's major depressive disorder, anxiety disorder and post traumatic stress disorder.

The patient reported that he has early morning awakening, lack of energy, as well as reduced ability to concentrate and focus. This has been increasing in severity not just for the last 3 months, but for years. His appetite is reduced and he suffers from anhedonia. The patient has panic attacks several times a week and even reported tremors due to his panic attacks. The patient fears how his wife and her family may take advantage of him by using his immigration situation as leverage. The patient reported that this takes a toll because it makes him fear he will be deported. The patient is anxiously awaiting the results of his three ongoing court cases.

The patient is also experiencing nightmares, flashbacks, avoidance and heightened startle reflex based on this his wife and her family taking advantage of him. He reported having all these symptoms 4-5 times per week to the extent that it negatively affects him being able to function and work.

The patient reported that in June-July 2016, a member of the patient's family requested to lease a Lexus under the patient's name. According to the patient, the family member said they would pay for the lease, but did not consistently do so and the responsibility of the payment fell on the patient. This is something the patient could not afford. The patient also reported that his wife would purchase superfluous things above their means with his money such as expensive clothes and jewelry despite the patient indicating that it was a lifestyle he could not support. The patient provides another example of this, when is was his wife's expectation and he would pay for a few of her relatives to stay in a five star hotel during a vacation in Niagara Falls. For example, on 4/2/17, the patient's wife coerced him to throw a lavish party at Astoria Manor for 200 people. It was $70 per plate. This was while the patient didn't have a job.

In January 13, 2017, the patient reported that his wife hit him in the eye. The patient filed a police report and has medical reports describing his injury. The patient reported that his wife demanded that he give money to her family/relatives.

In February 13, 2017 the patient and his wife had a joint immigration interview. The patient believes that his wife may have deliberately sabotaged the interview by not answering certain questions neither fully nor accurately. The patient suspects that his wife was feigning being unknowledgeable. For example, his wife was not able to state what the patient did for a living nor the color of their car. According to the patient, this was

in the absence of any substance use or abuse or lack of sleep. The patient believes that this was not because his wife was nervous, rather she wanted to sabotage this immigration interview.

In September 29, 2016, the patient was in a car accident. He was driving his taxi. The patient reported that the stress his wife put him in contributed to the accident. "I was constantly thinking about how she was tormenting me. She would call me constantly while I was driving and it was very distracting," said the patient. Following the accident, the patient only spent one night at St. Luke's hospital. According to the patient, he hit his teeth on the steering wheel and hurt his left should and left knee. No passengers nor pedestrians were injured. The patient reported his wife was unsupportive in his recovery process.

In February 2018 the patient's wife filed a restraining order for domestic violence against him. The patient clarified that he never hit his wife rather in 2017 his wife hit him. The patient reported that his friends witnessed his face being bruised and swollen after his wife hit him. The patient reported that his contributed to his depression and anxiety. The medical record presented by the patient seems to be consistent with the injury.

The patient became tearful several times in both our sessions because he was not able to visit his family in Bangladesh since he arrived in the US. The patient was tearful because his mother died in Bangladesh and he could not visit her. The patient also reported that being around his wife was so difficult that he would often sleep in his car outside to avoid facing her.

The patient is currently unemployed. However, used to work as a taxi driver. In 2016 he was in a car accident and subsequently followed up with a psychiatrist for 7 visits.

The patient denies hearing voices. Denies bipolar symptoms. Denies obsessive-compulsive symptoms. Deny suicidal and homicidal ideations. Denied ADHD. Denied ever going to jail. Novation also denied drugs and alcohol. He smokes 1 pack of cigarettes every 5 days.

Mental status exam. Calm, cooperative. Appropriately dressed. Fluent, spontaneous Bengali. Minimal English. Thought content: no suicidal nor homicidal ideations. Anxious from the stress his wife put him in. Thought process linear goal oriented. Insight and judgement good. The patient is motivated to get to the root of his depression and anxiety in order to resolve it.

Past psychiatric history. No inpatient psychiatric hospitalizations. After his car accident in 2016, the patient visited a psychiatrist six or seven times. He tried antidepressants such as Wellbutrin and Paroxetine in the past. As well as Ambien, Xanax and Klonopin. However, the patient is presently not taking ambien (nor is he prescribed it) and his Nightmares now are due to PTSD.

Medical history. Following the car accident in September 2016, the patient had chronic neck pain, chronic left shoulder and left knee pain. Medical medications include Atorvastatin 10 mg daily for hypercholesterolemia and omeprazole for GERD. Patient follows up regularly with his primary medical doctor.

Social history. The patient moved to New York City in 1998. The patient's mother died of a myocardial infarction in 2014, she was just 62 years old. However, the patient was not able to visit Bangladesh since he moved here in 1998 and he does not have any other family members in the US. The patient married in 2007, his wife has several family members living in the US.

The patient currently lives alone in Jackson Heights for the past 3 months. Prior to that he was living with his friend for one month.

The patient's wife had two previous marriages. The evaluation by the social worker in 2016 reveals that the patient's wife wants him around and not deported. Their lives are better for it. The patient had one previous marriage to a woman in Atlanta Georgia. The patient and his wife had a city hall marriage in 2007. The patient reports that his domestic emotional abuse began shortly after their marriage and began as early as 2008.

Impression, Discussion and Conclusions: [redacted] old man of [redacted] escent with major depressive disorder, panic disorder and post traumatic stress disorder.

This evaluation shows that there may be a correlation between the patient's depression, panic disorder and PTSD and the antagonism the patient experienced from his wife. The patient has experienced extreme emotional and physical distress due to the domestic emotional abuse and physical abuse. The patient's perception that his wife are antagonizing him may be more than just mere perception. Rather, it may be a legitimate threat to the patient and requires further investigation. More investigation is required to ascertain the Borderline, Narcissistic and Histrionic personality traits within the patient's wife and her family.

Further consideration should be given to the wife's Mental Health. The patient's wife was evaluated by a social worker who found that she was depressed. The patient demonstrated documentation proving this. The patient reported that his wife was prescribed medication but she became non-adherent with medication and follow-up.

The patient is considered by the writer to be a well-intentioned person and honest tax payor who was possibly abused by his wife and her family. The patient's depression, anxiety and PTSD were possibly caused entirely by the tumultuous relationship with his wife's family.

The patient and the writer chose to proceed with psychotherapy and medication management during our two sessions.

Regarding medications: Zoloft was started to help with depression, anxiety and PTSD and has been titrated up to 200mg. Klonopin 0.5mg daily was given first, but replaced with low dose Ativan 0.5mg bid prn since Klonopin (a long acting benzodiazepine) was making the patient sleepy during the day. He is also given Prazosin 5mg at bedtime to reduce nightmares of ptsd, with good effect.

The patient has been following up with me for medication management and psychotherapy monthly and has been showing improvement. There is a marked improvement in the patient since separating from his wife.

Please consider this as a treatment plan as well.